The C-R-Z Framework: Calibration, Reality-Return, and Zero-Velocity

A conceptual research note on a somatic-kinetic model, with an evidence audit and a proposed pilot

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Date: 13 November 2026
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> **CONCEPTUAL FRAMEWORK. This is not medical advice, not psychotherapy, and not a validated treatment.** C-R-Z is a personal somatic-kinetic model. Some of its parts line up with published findings. Some are plausible but untested as a package. Some are speculative and should be reframed or dropped before they are treated as mechanisms. Nothing in this note diagnoses, treats, or prevents any condition. If you are in crisis in the United States, call or text **988**. If you are in immediate danger, call **911**.

All credit and glory to God.

Abstract

This note records Ryan Espinosa's **C-R-Z Framework** as developed in three prior working papers: The Unified C-R-Z Somatic-Kinetic Framework; Returning to Reality and Neutralizing Threat Responses (V1 and V2); and Everyday C-R-Z: The Low-Friction Architecture. The model is meant to bypass slow verbal introspection, neutralize threat responses, and preserve personal sovereignty through three pillars — **Calibration (c)**, **Reality-Return (r)**, and **Zero-Velocity (z)** — and a brief action protocol, **Drop-Press-Snap**, with later daily adaptations.

The first task is to present that architecture faithfully. The second is to grade each major claim against literature actually checked for this draft, using three evidence tiers: **supported**, **plausible**, and **speculative**. The third is to cross-reference the framework with two related notes in this collection, Coherence Under Decay and the quantum boundary proposal, and with the Orun story's skill of dropping everything, listening for a key to reality, and exiting the gate. The note then proposes a small, testable within-subject pilot of Drop-Press-Snap against a control, on subjective distress and heart rate, and closes with safety limits, including the need for clinical follow-up when mania warning signs appear.

The short version of the audit is this. Outward sensory reorienting, affect labeling, plantar contribution to posture, and slow, exhale-emphasized breathing have real literatures behind them. A brief motor task can interrupt rumination, but not specifically because an ASL “C” handshape bypasses the phonological loop. Shifting weight onto the soles is a reasonable proprioceptive anchor; a “solar plexus reflex point” on the foot is not a supported physiological mechanism. Expansive-posture hormone claims failed to replicate, though modest effects on how a posture feels are still discussed. “Glimmers,” in Deb Dana’s polyvagal-informed usage, are small cues of safety and must not be confused with mania prodromes. Polyvagal theory itself is disputed. C-R-Z agrees with Coherence Under Decay and with the Orun story on returning to a rest that is not empty and on leaving a loop. It conflicts with them if Zero-Velocity is read as a literal arrival at v = 0, and it conflicts with itself if “mania glimmers” and safety glimmers keep the same name.

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0. Reader’s guide and evidence tiers

C-R-Z is the author’s framework. This note does not replace the three source papers. It is a companion audit: keep the model, credit the people whose work it touches, and say plainly where a postulate is ahead of the evidence.

Each claim below is tagged as one of:

- **Supported.** A real, identified literature backs the narrow claim. Support is never a claim that C-R-Z as a whole has been tested.
- **Plausible.** The claim is a reasonable extension of nearby findings, but the specific C-R-Z move, label, or bundle has not been shown to work as described.
- **Speculative.** The claim goes beyond current evidence, rests on a weak or disconfirmed tradition, or is best read as metaphor or a values statement.

A fourth label, **[Values]**, is used where the claim states what is worth protecting (sovereignty, non-extraction, grace, the dignity of leaving). Values are not tested by experiment and are not diminished by being labelled.

Sources in Section 10 were checked on 13 November 2026 against publisher, PubMed, PMC, or other bibliographic records and against abstracts or open full text returned in those records. They were not all read in their entirety. Page numbers and DOIs are given where the record supplied them.

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1. Source of the framework

C-R-Z is drawn from three of the author’s working papers:

1. The Unified C-R-Z Somatic-Kinetic Framework

2. Returning to Reality and Neutralizing Threat Responses, including its V2 expansion
3. Everyday C-R-Z: The Low-Friction Architecture

In the author's wording, the framework is a "hardware-level" somatic-kinetic model designed to bypass slow verbal introspection, neutralize threat responses, and preserve absolute personal sovereignty. "Hardware-level" is treated here as the author's metaphor for a fast, low-verbal loop through posture, pressure, and attention, not as a claim about computer hardware or a newly measured neural circuit.

This note is the first time the framework is written up in the MindFrame research collection, alongside Coherence Under Decay (12 November 2026) and the quantum boundary proposal (12 November 2026).

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2. The framework, presented faithfully

2.1 The three pillars

Calibration (c). Instantaneous mental and somatic tuning that uses minimal alphanumeric notation or ASL-mediated motor cues, such as an ASL "C" handshape, in order to bypass phonological-loop overhead and halt internal rumination.

Reality-Return (r). Immediate re-engagement with external sensory input, labeling of the environment, and attentional reorienting, in order to pull awareness out of internal threat simulations.

Zero-Velocity (z). Gravitational grounding, written $v \rightarrow 0$, through plantar mechanoreceptor stimulation. The aim is to establish boundary sovereignty, halt fight-or-flight kinetic momentum, and prevent "negative perpetual motion."

The three letters are meant to be used together, not as a menu of optional moods. Calibration marks and interrupts the inner loop. Reality-Return aims attention at what can be verified outside the body of the thought. Zero-Velocity stills the body's forward threat-momentum by putting weight into contact with the ground.

2.2 The Drop-Press-Snap protocol

For acute threat or conflict, the source papers specify a split-second sequence:

1. **The Drop.** Instantly drop the shoulders and shift all body weight downward into the heels and soles against the earth.
2. **The Press.** Subtly press the big toe downward and apply pressure to the "solar plexus reflex point" on the sole, with the stated aim of triggering vagal down-regulation.
3. **The Snap.** Execute a micro-sign against the leg (the ASL "C" handshape) while anchoring attention externally to reality.

The protocol is intended to be brief enough to use in the middle of a conversation or a surge of alarm, without first narrating the feeling at length.

2.3 V2 and daily adaptations

The V2 expansion and the everyday, low-friction architecture add:

- **Breath mechanics:** inhaling “internal friction” and exhaling “collective peace.”
- **The Unclenched Hand:** an uncrossed, open-hand posture.
- **A soft triangle gaze** with a “transparent glass-pane filter.”
- **Early-warning detection** that tells triggers apart from “mania glimmers.”

These are daily or preparatory habits, not only crisis moves. They are meant to lower the friction of using C-R-Z so that the protocol does not itself become another rumination.

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3. Evidence audit of each claim

3.1 Summary table

Claim in C-R-Z	Tier	Short verdict
Outward sensory reorienting and environmental labeling pull attention out of an inner threat simulation	**Supported** (as a family of clinical practices; the exact C-R-Z script is untested)	Lines up with grounding and orienting work in trauma care and with affect-labeling research. The popular 5-4-3-2-1 countdown itself has little direct trial evidence.
A brief, structured return to present sensory facts can be used when distress or dissociation rises	**Supported** as clinical practice, **plausible** as a stand-alone C-R-Z effect	Used as an antidissociative skill in DBT-related trauma work; not shown as a packaged C-R-Z protocol.
Rumination uses verbal working memory (the phonological loop)	**Supported** as a model of verbal short-term memory; **plausible** as an account of verbal rumination	Baddeley’s model is standard. Treating rumination as inner speech is reasonable but not settled.
An ASL “C” (or minimal alphanumeric cue) bypasses phonological-loop overhead and halts rumination	**Speculative** in that specific form; **plausible** as a generic motor interrupt	Articulatory suppression is a real method, but it usually uses speech-like action. A 2022 study found both silent mouthing and finger tapping reduced induced rumination, with only a weak, uncertain advantage for mouthing.
Plantar input helps the standing body know where it is	**Supported**	Cutaneous mechanoreceptors in the sole contribute to postural control.
Shifting weight into the heels and soles can serve	**Plausible**	

Claim in C-R-Z	Tier	Short verdict
as a present-moment anchor		Follows from the plantar literature and from sensory grounding practice. Not tested as "Zero-Velocity."
A "solar plexus reflex point" on the sole triggers vagal down-regulation	**Speculative** (and, as organ-specific reflexology, unsupported)	Systematic reviews find no convincing evidence that reflexology is an effective medical intervention, and no support for organ-specific foot maps. Reframe the press as a proprioceptive attention anchor.
v → 0 literally halts fight-or-flight momentum and stops "negative perpetual motion"	**Speculative** if read as physiology or physics	Useful as a metaphor for coming to rest. Conflicts with Coherence Under Decay if taken as a perfect zero.
Dropping the shoulders and opening the hands changes threat physiology in a unique, reliable way	**Plausible** for modest felt effects; **speculative** for hormone or "power" mechanisms	Power-posing hormone and risk-taking effects failed a large replication. Later work finds more support for how postures feel than for downstream biology. Upright versus slumped posture has some mood and stress-task evidence.
Slow or exhale-emphasized breathing can lower arousal and improve mood	**Supported**	A 2018 systematic review of slow breathing, and a 2023 randomized comparison of brief daily breath practices, support mood and arousal effects. The 2023 study did not find changes in resting heart rate or HRV over a month.
Inhaling "internal friction" and exhaling "collective peace" is a distinct mechanism	**Speculative** as mechanism; **[Values]** as wording	The pacing may do the work. The phrases state an aim, not a measured pathway.
Soft triangle gaze with a glass-pane filter is a distinct, effective technique	**Speculative** as a named method; **plausible** as outward visual anchoring	No identified trial of this exact form. It can be grouped with attentional deployment and environmental orienting.
"Glimmers" as small cues of safety, distinct from triggers	**Plausible** as a clinical heuristic; the polyvagal theory around it is **disputed**	Deb Dana's term is useful if kept ordinary and phenomenological. It should not be treated as a validated autonomic marker.
"Mania glimmers" as early-warning signs	**Naming is speculative and risky**; the underlying clinical idea is **supported** if relabelled	Early-warning work in bipolar disorder is real and important. Dana's "glimmers" are cues of safety, almost the opposite of a manic prodrome. The two must not share a name.
Drop-Press-Snap, as a three-step package, neutralizes threat and preserves sovereignty	**Plausible** as a brief coping sequence worth testing; **speculative** as a proven architecture	No identified trial of the package. Sovereignty is a **[Values]** aim, not a measured outcome, unless operationalized (for example, as the freedom to stop and leave).

3.2 Reality-Return: grounding, orienting, and labeling

Reality-Return is the pillar with the closest neighbors in existing care. Trauma-informed and dialectical-behavior-therapy (DBT) practice often asks a person in rising distress or dissociation to name what can be seen, touched, or heard in the room, so that attention leaves an inner replay and returns to a shared present. A 2025 paper that set out to build an operational definition of therapeutic grounding describes the shared core as engagement with basic sensory input, and sometimes with simple cognitive recall, in order to shift focus away from distressing internal stimuli (Hammond and Brown 2025). In DBT-PTSD programs, antidissociative and sensory-grounding skills are used so that exposure work can proceed; one feasibility study of a computer-assisted aid found that patients often rated such skills as useful for blocking dissociation (Görg et al. 2016). That is practice-level support for the family of methods, not a trial of C-R-Z.

The popular 5-4-3-2-1 countdown (five things seen, four heard, and so on) is a convenient script inside that family. No dedicated randomized trial of that exact sequence was identified for this draft. It should be cited as a widely taught clinical tool, not as a proven protocol.

Environmental labeling also touches a more specific finding. Lieberman, Eisenberger, Crockett, Tom, Pfeifer, and Way (2007) reported that putting feelings into words — affect labeling — reduced amygdala and other limbic responses to negative images, with increased right ventrolateral prefrontal activity, relative to other forms of encoding. C-R-Z's labeling is slightly different: it labels the room more than the feeling. The overlap is still real. Naming a chair, a window, or "this is a thought, that is the river" is a low-demand act of putting experience into words and pointing at something present. That is **supported** as a nearby mechanism, and **plausible** as the reason Reality-Return may work, pending a direct test.

Two cautions belong next to this support. First, for some people with trauma histories, turning attention inward onto bodily sensation can briefly raise arousal. Reality-Return is already written as an outward move, and it should stay that way. Second, grounding is a coping skill, not a treatment for PTSD, panic, or dissociation.

3.3 Calibration: the phonological loop, rumination, and the ASL "C"

Calibration rests on a standard model and on a much more specific hypothesis.

The standard model is **supported**. Baddeley and Hitch (1974) described working memory as a limited workspace with a verbal rehearsal loop. In later statements of the model, the phonological loop has a short-lived store and an articulatory rehearsal process; saying an irrelevant word out loud or silently ("articulatory suppression") impairs verbal short-term recall (Baddeley, Lewis, and Vallar 1984; reviewed in Baddeley 2012). If a stretch of rumination is carried as inner speech, then occupying the rehearsal process could, in principle, make that speech harder to keep going.

The specific hypothesis — that a minimal alphanumeric cue or an ASL "C" pressed against the leg bypasses phonological-loop overhead and thereby halts rumination — is **speculative**. Articulatory suppression, in the laboratory, is speech-like: repeating "the," mouthing, or otherwise engaging the articulators. A handshape against the thigh is a tactile and manual act. It may occupy attention, mark a decision, or interrupt a chain of thought. It is not, on present evidence, a way of taking the phonological loop offline.

A direct test of the nearby idea is already more modest than the C-R-Z wording. Nalborczyk, Perrone-Bertolotti, Baeyens, Grandchamp, Spinelli, Koster, and Lœvenbruck (2022) induced rumination in 106 adults and then had them spend five minutes on silent mouthing or on finger tapping. Self-reported rumination fell after both tasks. The advantage for articulatory suppression over tapping was only slight and uncertain. If that pattern holds, Calibration may work because a brief, deliberate motor act interrupts the loop, not because the ASL "C" is a privileged linguistic key. That is still useful. It just recommends humility about the handshape, and it licenses a seated or alternative cue for people who cannot or do not use that sign.

Minimal alphanumeric notation, used as a private shorthand instead of a long inner monologue, is likewise **plausible** as a way to reduce verbal load, and **speculative** as a complete bypass of introspection. Implementation intentions — a short, pre-decided "if-then" cue — have their own literature (Gollwitzer 1999). C-R-Z can borrow that structure without claiming a new memory-system effect.

3.4 Zero-Velocity: plantar input, the reflexology problem, and v → 0

Two claims need to be separated.

The first is **supported**. The sole of the foot is densely equipped with cutaneous mechanoreceptors. Kavounoudias, Roll, and Roll (1998) argued that the plantar sole behaves as a "dynamometric map" for human balance: stimulating different regions of the sole produces direction-specific postural responses. They later showed that foot-sole and ankle-muscle inputs contribute jointly to erect posture (Kavounoudias, Roll, and Roll 2001). A later review of single-unit recordings describes the innervation of the human foot sole in detail (Strzalkowski, Peters, Inglis, and Bent 2018). Shifting weight into the heels and soles, and pressing the big toe, will change that afferent pattern. As a way of "landing" in the present, this is **plausible** and physically real.

The second claim is **speculative** and should be withdrawn as a mechanism. The source papers ask the person to press a "solar plexus reflex point" on the sole in order to trigger vagal down-regulation. That wording comes from reflexology, which maps patches of the foot onto distant organs. Ernst, Posadzki, and Lee (2011), updating an earlier systematic review of randomized trials, concluded that the evidence does not demonstrate that reflexology is an effective treatment, and the tradition's

organ maps remain unvalidated assumptions (see also Ernst 2009). There is no identified physiological pathway by which a named point on the sole would selectively regulate the solar plexus or the vagus in the manner claimed.

Reframe, which the author requested in spirit by asking that postulates be cross-checked before they enter the papers: keep the toe and sole press; drop the organ-specific point and the guaranteed vagal claim. Describe the press as a proprioceptive and tactile attention anchor — a way of feeling the ground on purpose. If vagal or heart-rate effects appear in a pilot, report them as outcomes, not as already-known results of a reflex point.

The notation $v \to 0$ should be treated the same way. As a picture of coming to rest, it is clear. As physics or as a claim that fight-or-flight "kinetic momentum" is halted and "negative perpetual motion" is prevented, it is **speculative**. Bodies do not reach zero motion while alive and standing. Coherence Under Decay already insists that no real system reaches perfect isolation or perfect zero, and that a rest state should be reachable, honest, and never empty. Zero-Velocity is at its best when it means approach rest; put weight into the ground; do not keep accelerating the inner chase.

Dropping the shoulders can be kept as a simple release of upper-body bracing. It does not need a larger posture theory under it.

3.5 Posture: the Unclenched Hand, and what did not survive replication

V2's Unclenched Hand — uncrossed posture, open hands — should be stated modestly.

Carney, Cuddy, and Yap (2010) reported that brief "power poses" changed felt power, risk-taking, testosterone, and cortisol. Ranehill, Dreber, Johannesson, Leiberg, Sul, and Weber (2015), in a larger, experimenter-blind replication (N = 200), reproduced an effect on self-reported feelings of power but found no effect on hormones or risk-taking. Simmons and Simonsohn (2017) argued, with a p-curve analysis, that the evidential value of the broader power-posing literature was weak. Later replies and meta-analyses have been more favorable to effects on felt power and some affective ratings than to the original endocrine and behavioral package (Gronau et al. 2017; Cuddy, Schultz, and Fosse 2018; Elkjær, Mikkelsen, Michalak, Mennin, and O'Toole 2022). The honest summary is: **do not rest C-R-Z on power-posing physiology**. An open, uncrossed stance may still be a decent, low-cost cue of not bracing for a fight. That is **plausible** as comfort and self-signal, **speculative** as a unique regulator of threat circuitry.

A separate posture literature is slightly more useful here. Nair, Sagar, Sollers, Consedine, and Broadbent (2015) found that upright versus slumped seated posture affected mood, self-esteem, and responses during a stress task. That is not the Unclenched Hand, and it is not a license to overclaim, but it supports the general idea that how one sits or stands can color how a stressful moment feels.

3.6 Breath: what is supported, and what is a prayer in the breath

Slow breathing, often near six breaths per minute, has been associated with higher heart-rate variability and with reports of greater relaxation and lower anxiety in healthy adults, though the primary studies are often small and unevenly reported (Zaccaro, Piarulli, Laurino, Garbella, Menicucci, Neri, and Gemignani 2018). Balban, Neri, Kogon, Weed, Nouriani, Jo, Holl, Zeitzer, Spiegel, and Huberman (2023), in a remote randomized study (NCT05304000), compared five minutes a day of cyclic sighing (exhale-emphasized), box breathing, or cyclic hyperventilation with retention against mindfulness meditation, over one month. Breathwork, especially cyclic sighing, improved positive affect and reduced respiratory rate more than meditation. Resting heart rate and heart-rate variability did not change significantly over the month. That last point matters for any pilot that uses heart rate as a primary outcome: mood and respiratory rate may move when HR and HRV do not.

C-R-Z's own wording — inhale internal friction, exhale collective peace — is **[Values]** language riding on a **supported** class of practices. The phrases are a way of aiming the breath. They should not be presented as the mechanism. If a daily adaptation is written for others, pair the author's phrases with a simple, testable instruction (for example, a slightly longer exhale than inhale, for a few minutes), and let the phrases remain what they are: an intention, not a result.

3.7 Soft triangle gaze and the glass-pane filter

No trial of a "soft triangle gaze" with a "transparent glass-pane filter" was identified. The move can be read, charitably, as: rest the eyes on three outward points, keep the gaze soft, and imagine a little distance, as if looking through glass, so that attention stays in the room without staring or scanning for threat. As a form of outward attentional deployment, that is **plausible**. As a named, distinctive technique with special effects, it is **speculative**. In an everyday architecture it can stay, described in ordinary words, and should be dropped from any list of established mechanisms until someone tests it.

3.8 Early warning: triggers, glimmers, and mania

This is the place where careful naming is part of safety.

Triggers, in ordinary clinical speech, are cues that reopen threat, memory, or craving. Learning to notice them early is a standard aim of many therapies and of relapse-prevention work.

Glimmers, in Deb Dana's polyvagal-informed writing, are small, ordinary cues of safety and connection — a patch of light, a kind voice, a moment of ease — that can be noticed on purpose (Dana 2020). As a practice of attending to what is already bearable, this is **plausible** and compatible with Reality-Return. The theoretical wrapping is weaker. Polyvagal theory, as advanced by Porges (for example 2007), has been sharply criticized on physiological and comparative grounds. Grossman (2023) ar-

gues that several of its basic premises, including the reading of respiratory sinus arrhythmia as a straightforward index of a distinct “smart vagus,” do not hold. C-R-Z can keep the habit of noticing small, real, present cues of safety without committing itself to a disputed autonomic architecture. Say that plainly.

Mania early-warning signs are a different object, and here the literature is **supported**. People with bipolar disorder often have a personal prodrome — reduced sleep, more talk, rising energy, irritability, racing thoughts, more spending, or more goal-directed activity — days to weeks before a manic or hypomanic episode (reviewed by various clinical groups; see also Perry, Tarrier, Morriss, McCarthy, and Limb 1999). A Cochrane review found that interventions to help people recognize early signs of recurrence can delay bipolar relapse (Morriss, Faizal, Jones, Williamson, Bolton, and McCarthy 2007). That work should be credited, used, and not renamed lightly.

The source papers’ phrase **“mania glimmers”** is a conflict of terms. Dana’s glimmers are cues of safety. Manic prodromes are cues that a person may be leaving ordinary range and may need a clinical plan, sleep protection, medication review, or contact with a clinician. Treating a manic early-warning sign as a glimmer risks exactly the misreading the V2 adaptation is trying to prevent: taking a rising, glittering, high-energy state for a sign that one is “coming back to life.” The recommended reframe is:

- keep **glimmers** for small, steadying cues of present safety, or drop the word if it confuses;
- keep **early-warning signs** or **prodromes** for mania and hypomania;
- teach, where relevant, that the two can be told apart in part by sleep, duration, impairment, and whether other people are worried;
- treat mania warning signs as a reason for clinical follow-up, not as a prompt to run Drop-Press-Snap harder.

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4. Cross-references

4.1 Inside C-R-Z

The three pillars hang together if they are read as one move: interrupt the inner monologue, point attention at the present, and put the body into contact with the ground. They pull apart if each pillar is asked to do unique physiological work that the others do not. Calibration and the Snap overlap (both are brief motor cues). Reality-Return and the last beat of the Snap overlap (both aim attention outward). Zero-Velocity and the Drop overlap (both put weight into the ground). That overlap is a strength for a low-friction practice — one short sequence, not three therapies — and a warning against claiming three independent mechanisms.

There is a deeper internal tension between “bypass verbal introspection” and Reality-Return’s use of labeling. Lieberman’s finding is that words can help. C-R-Z can resolve this by distinguishing rumination (repetitive, closed, self-referential speech) from labeling (short, outward, present-tense naming). The framework already wants fewer words, not no contact with language.

A second tension is between absolute sovereignty and the everyday architecture’s interest in “collective peace.” Sovereignty, in the better reading, is the right to stop, to refuse, and to leave. It is not a requirement to regulate alone. The Orun story, discussed below, is explicit that a living voice can snap a person back. That is a friend of Reality-Return, not a threat to it.

4.2 Coherence Under Decay and the quantum boundary proposal

Coherence Under Decay (Espinosa, with Grok Bot, 12 November 2026) joins the quantum boundary proposal to the Zero-Point chronomimetic notes under one control template: keep a system useful and coherent while decay, noise, and idle coupling pull on it; use bounded, gentle control; keep a protected rest state that is reachable, honest, and never empty. The quantum boundary proposal (same date) maps Ariel labels onto textbook qubit results and refuses to treat a metaphor as a gate voltage. Both notes are conceptual. They are not new physics.

Where they agree with C-R-Z:

- A return to rest is not a collapse into a void. Zero-Velocity, the Sanctuary in the economic notes, and Orun’s teaching that rest is not emptiness are the same shape.
- Bounded, brief control is preferred to grinding effort. Drop-Press-Snap is a short intervention, not a campaign against the self. Coherence Under Decay likewise rejects fighting through as the only path and warns against compulsion mechanics.
- Honest naming matters. The quantum note refuses metaphorical overclaim; this audit should do the same for reflex points, $v = 0$, and polyvagal certainty.
- Clean egress is part of dignity. Reality-Return and “exit the gate” are kin to the app’s single-tap exit and to the rule that rest must remain leaveable.
- Safety monitors should offer reflection and never punish. C-R-Z’s early-warning idea belongs in that family, provided mania signs are routed to care rather than to a self-score.

Where they conflict, or will conflict if wording is not tightened:

- **Perfect zero.** C-R-Z’s $v \to 0$, read literally, collides with the physical and design lesson of Coherence Under Decay: no system reaches perfect isolation or perfect zero; a friction floor and a residual decay rate are features, not failures. The repair is already given in Section 3.4: Zero-Velocity is an approach to rest, not an arrival at nothing.

- **Mechanism versus template.** Coherence Under Decay is careful to call its bridge a structural analogy. C-R-Z sometimes speaks in “hardware-level” and vagal-guarantee language. The CUD standard should win: say what is analogy, what is design, and what would count as a measurement.
- **Isolation and sovereignty.** If Zero-Velocity’s “boundary sovereignty” is taken to mean withdrawing from other people, it cuts against the Circuit of Grace and against the finding, in the author’s own fiction, that relationships snap a person back. Sovereignty is better defined as not being overwritten, and as the right to leave, not as a closed system.

The quantum layer does not supply evidence for C-R-Z, and C-R-Z does not supply evidence for qubit control. Any shared vocabulary (gate, zero, friction) is analogical. That limit should stay visible.

4.3 The Orun story: drop everything, listen for a key, exit the gate

In Selu Cannot Buy the Morning Back and the game The Path of Rivel, Neru’s knowledge-seeking and her imagined conversations with the dead can close into a loop. The skill the story finally asks for is to notice that loop, put down the purse and the maps, and listen for one real thing in the present — breath, the river, a living voice. That is the key that lets her exit the gate. The game gives this a meter: when the loop rises, a “Drop everything and listen” choice appears; it clears the loop, restores calm, and is required for the best ending. A later line in that work states the human version directly: relationships snap you back in reality.

Agreement is close, and it is the best literary cross-check the framework has.

- Calibration is the noticing: the loop is looping; stop adding turns.
- Reality-Return is the listening for a key that is not another inner conversation.
- Zero-Velocity is the putting-down: weight off the chase, into the ground, hands empty of the maps.
- Drop-Press-Snap is a bodily spelling of “drop everything and listen.”
- V2’s outward gaze and open hands match the story’s refusal to keep bargaining with a closed inner room.

The story also corrects the framework in two places. First, the key is often a living other person, not only a plantar signal. A C-R-Z practice that cannot make room for a voice, a touch, or a shared commons is narrower than the author’s own narrative. Second, the story will not let rest become a loophole, and it will not let knowledge bought from the dead replace consent and relationship. That is the same warning Coherence Under Decay gives the Sanctuary: a rest state that becomes a shelter for hoarding, rumination, or refusal of the living world is no longer rest.

There is no serious conflict of aim, only a conflict of emphasis if C-R-Z is practiced as a closed, solo circuit.

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5. A simple, testable pilot

C-R-Z has not been tested as a package. A first study should be small, pre-registered, and willing to fail.

Design. A within-subject laboratory or quiet-room comparison, in randomized order, of Drop-Press-Snap versus a control after a mild, ethically acceptable stress or worry induction (for example, a brief worry period or a standard mild cognitive stressor). This first pilot should not use trauma-script exposure. A third arm, if the sample allows, could be an established brief comparison such as a minute of 5-4-3-2-1 or of exhale-emphasized breathing, so that the question is not only "better than sitting still?" but "different from its nearest neighbor?"

The C-R-Z arm, for the pilot, should use the reframed protocol: drop the shoulders and put weight into the heels and soles (or into the feet if seated); press the big toe and the ground, described only as a tactile anchor, with no reflexology map; make the agreed motor cue (ASL "C" against the leg, or a simple agreed alternative) and name, quietly, two or three things in the room. The author's original wording can be taught as the personal form of the practice, but the tested instructions should not include unsupported organ claims.

Control. Sit or stand still for the same number of seconds, with no special cue, or count backward as a nonspecific cognitive control. The protocol's duration should be fixed in advance (for example, 30–60 seconds).

Primary outcomes. (1) Change in subjective distress, on a 0–100 visual analogue scale or a Subjective Units of Distress rating, from post-induction to post-technique. (2) Change in heart rate over the same window.

Secondary outcomes. Respiratory rate; if a device is already in place, heart-rate variability, clearly labelled secondary in light of Balban et al. (2023); a short state-anxiety scale; a one-item "how present do you feel?" rating; and, if Calibration is a target, a one-item rumination rating. Qualitative comments on whether the cue felt usable in the moment.

Sample and ethics. A feasibility sample (for example, a few dozen adults). Informed consent. Ethics or IRB review if the results will be published or if a clinical population is recruited. Exclude people in acute crisis, uncontrolled psychosis, or current manic or mixed episodes. People with bipolar disorder who are stable may take part only with a pre-specified plan and, where appropriate, clinician awareness; mania warning signs during the study are a stop rule, not more protocol. Offer a seated version for anyone with balance risk. Data private by default.

What would support the design, as feasibility evidence only. A pre-specified, modest reduction in distress after Drop-Press-Snap relative to control, with or without a short-lived heart-rate change, without serious adverse events, and with most participants able to complete the sequence. Effects should be reported with uncertainty, not as proof that C-R-Z "works."

What would count against it. No difference from control on distress; distress that worsens for more than a rare, transient case; heart-rate claims that were promised in advance and do not appear (in which case drop or demote the physiological story); participants finding the reflexology language confusing or untrustworthy; or any signal that people with rising manic signs used the protocol instead of seeking care.

This is a first measurement, in the same spirit as the small app pilot in Coherence Under Decay, Section 9. It is not a registration of a treatment.

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6. Safety

C-R-Z is a personal practice framework. **It is not medical treatment, not a substitute for psychotherapy or medication, and not a crisis service.**

- In the United States, if you are in emotional crisis or thinking about suicide, call or text **988**. If you are in immediate danger, call **911**.
- If distress, panic, dissociation, or despair is severe, persistent, or worsening, contact a clinician or a local emergency service.
- **Mania or hypomania warning signs** — including markedly less need for sleep, a sustained burst of energy or confidence that others find unlike you, racing speech or spending, or a rising sense that ordinary limits no longer apply — warrant clinical follow-up. They are not "glimmers" to be ridden, and they are not a reason to intensify Drop-Press-Snap in place of care.
- People with trauma histories should keep the outward emphasis of Reality-Return. If a turn toward the body increases alarm or detachment, stop.
- The weight-shift in the Drop should not be done in a way that risks a fall. A seated version, with both feet on the floor, is a full version, not a lesser one.
- Nothing in this note is an instruction to leave prescribed medication or to delay emergency care.

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7. What this is not

- Not a validated treatment, diagnosis, or medical device.
- Not evidence that an ASL "C," a foot reflex point, or a triangle gaze has a unique neural pathway.
- Not a confirmation of polyvagal theory, of reflexology, or of power-posing physiology.

- Not a claim that economics, quantum mechanics, or fiction prove a somatic protocol. Cross-references in Section 4 are structural and narrative, not empirical transfer.
- Not a promise that threat can be neutralized on demand, or that sovereignty can be made absolute.
- Not a reason to ignore other people when a living voice is the key that works.

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8. Limitations

This audit is only as strong as the records checked on 13 November 2026. Some nearby literatures (for example, sensorimotor psychotherapy and somatic experiencing) were not reviewed in depth and should not be treated as silently endorsed or silently dismissed. The three source papers were not available to this draft as public, dated, paginated documents; the framework is taken from the author's own synopsis, which he offered as the authoritative short form. If a later comparison with the full source papers shows a missed nuance, the source papers govern the description, and this audit should be revised.

The evidence tiers grade component claims, not the character of the author or the worth of the values layer. A speculative mechanism can sit next to a sound aim. The aim is not thereby proved, and the person is not thereby discredited.

A within-subject pilot will not settle long-term use, clinical populations, or the daily V2 adaptations. Demand characteristics are likely: people who believe in a sequence will often report that it helped. Pre-registration, a credible control, and at least one physiological measure that can fail are partial guards, not complete ones.

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9. Recommendations

1. Keep the three-part shape: interrupt, look outward, come to rest on the ground.
2. Keep Drop-Press-Snap as a brief sequence, with the Press reframed as a toe-and-sole attention anchor, and with a documented alternative to the ASL "C."
3. Keep the breath, the open hands, and a soft outward gaze, described in ordinary language, without special-mechanism claims.
4. Replace "mania glimmers" with "mania early-warning signs" or "prodromes," and keep "glimmers," if at all, for small cues of present safety.
5. Write Zero-Velocity as an approach to rest (v approaching a floor, not v = 0), in line with Coherence Under Decay.
6. Let relationship stay inside Reality-Return: a living voice is a valid key.
7. Run the pilot in Section 5 before any public claim that the package regulates the vagus, hormones, or threat "at the hardware level."

8. Leave the values — sovereignty, grace, the right to leave, collective peace as an exhaled hope — labelled as values.

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10. References

10.1 Source papers for the framework

1. Ryan Espinosa, The Unified C-R-Z Somatic-Kinetic Framework, working paper.
2. Ryan Espinosa, Returning to Reality and Neutralizing Threat Responses, V1 and V2, working papers.
3. Ryan Espinosa, Everyday C-R-Z: The Low-Friction Architecture, working paper.

The synopsis of pillars, Drop-Press-Snap, and V2 adaptations used in this note was provided by the author on 13 November 2026.

10.2 Related notes in this collection

1. Ryan Espinosa, with Grok Bot, Coherence Under Decay: A Shared Control Template for Qubits and a Zero-Debt Commons, 12 November 2026, MindFrame research notes, Coherence-Under-Decay.
2. Ryan Espinosa, with Grok Bot, Dynamic Boundary Control and Vacuum-Referenced Stabilization of Qubits: A conceptual research proposal for Project Ariel (revised), 12 November 2026, Quantum-Boundary-Proposal.
3. Ryan Espinosa, Selu Cannot Buy the Morning Back and The Path of Rivel (Orun narrative and game, 2026), including the skill of dropping everything, listening for a key to reality, and exiting the gate.

10.3 Literature checked for this draft

1. A. D. Baddeley and G. J. Hitch, “Working memory,” in G. H. Bower (ed.), The Psychology of Learning and Motivation, Vol. 8 (Academic Press, 1974), pp. 47–89.
2. A. Baddeley, “Working memory: theories, models, and controversies,” Annual Review of Psychology **63**, 1–29 (2012).
3. A. Baddeley, V. Lewis, and G. Vallar, “Exploring the articulatory loop,” Quarterly Journal of Experimental Psychology **36A**, 233–252 (1984). Bibliographic details for this item should be spot-checked against the publisher record if the paper is quoted at length; the articulatory-suppression effect itself is standard in Baddeley’s later reviews.
4. L. Nalborczyk, M. Perrone-Bertolotti, C. Baeyens, R. Grandchamp, E. Spinelli, E. H. W. Koster, and H. Lœvenbruck, “Articulatory suppression effects on induced rumination,” Collabra: Psychology **8**(1), 31051 (2022). https://doi.org/10.1525/collabra.31051

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9. A. Kavounoudias, R. Roll, and J.-P. Roll, “Foot sole and ankle muscle inputs contribute jointly to human erect posture regulation,” Journal of Physiology **532**(3), 869–878 (2001).
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11. E. Ernst, “Is reflexology an effective intervention? A systematic review of randomised controlled trials,” Medical Journal of Australia **191**(5), 263–266 (2009).
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17. A. J. C. Cuddy, S. J. Schultz, and N. E. Fosse, “P-curving a more comprehensive body of research on postural feedback reveals clear evidential value for power-posing effects: reply to Simmons and Simonsohn (2017),” Psychological Science **29**(4), 656–666 (2018).
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Item 9 carries a flag to confirm pagination against the publisher version before any formal quotation. All other bibliographic details above were taken from publisher, PubMed, PMC, or Cochrane records checked on 13 November 2026, or from abstracts and open text supplied by those records. They were not all read in full.

10.4 AI assistant

1. Grok Bot (2026). AI assistant used, under the author’s direction, to help draft this note, look up and format literature, cross-reference the author’s related papers, and prepare the PDF. Grok Bot is an AI assistant. It is not a co-investigator, not a clinician, and not a source of primary evidence. Every empirical claim in this note is only as strong as the published record it cites.

All credit and glory to God.